PFVR - Participant Follow-up Visit Record

Visit identification, purpose, components and timing

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Study / protocol	CIP version	Site / code	PFVR ID
GLENOS-01 / GS-2026-01	CIP v4.2	Riverside Orthopedic Center / 014	PFVR-014-002-M3
Participant code	**Visit (name / nominal day)**	**Index procedure date (ICDL ID)**	**Controlled location**
014-002	3 months / day 90	8 Jul 2026 (ICDL-014-002)	ISF tab 6 (Follow-up) / PFVR-014-002-M3

A Purpose and use

The PFVR records one participant's one follow-up visit: when it happened (C), each component the CIP requires with its date, completion and timing (D), whether images and tests are usable (E), what the participant reports (F), the clinical assessment (G) and who did it (H). Problems that need a response go to I, and the visit is classified in J.

A component done on a later contact for the same visit stays in this record with its own date. Write the short finding that matters and cite the clinical source, the Image Quality Log (IQL) or the SART for the rest.

Unit of analysis	Decision supported	Inputs	Outputs
One participant and one follow-up visit of the CIP.	Whether the visit is complete and usable, usable with a limit, or needs follow-up.	CIP visit schedule, clinical source, questionnaires, IQL, earlier PFVR.	Visit outcome (J) and component counts for the Follow-up Compliance Review (FUCR); exceptions and routes (I).

B Applicable visit component guide

The CIP visit schedule defines the components; tick the families and examples that apply to this visit, then list each one in D.

Component family	Examples that may apply	Recorded in
Imaging and objective tests	☑ X-ray ☐ CT ☐ MRI ☐ Ultrasound ☐ Laboratory ☐ Other test	D: done, date, window · E: usable?
Participant-reported outcomes	☑ Pain ☑ Function ☑ Instability ☐ Swelling ☐ New symptom ☐ Satisfaction	D: each instrument · F: change reported
Clinical and functional assessment	☐ Examination ☑ Range of motion ☐ Strength ☑ Stability ☑ Wound ☑ Functional score	D: done · G: result
Care and safety review	☐ Physiotherapy ☐ Medication review ☑ Safety review ☐ Referral	D: done · I: response · SART if an event

C Visit timing and contacts

This section records when the visit happened. Whether it is complete and usable is decided in J from D and E.

Nominal target date (reference event)	Window: first and last day (CIP rule)	Actual visit date	Days from target
6 Oct 2026 (day 90 after surgery)	22 Sep to 20 Oct 2026 (day 90 ± 14)	2 Oct 2026	-4

Contact	Date · mode	Day from index	Components done (# in D)	Purpose · source reference
First	2 Oct 2026 · in person	86	1, 3 to 6; 2 in part	Clinic visit; ASES page 2 not done (participant had to leave). Clinic note CN-014-002-M3
Return	9 Oct 2026 · in person	93	2	ASES page 2 completed in clinic; same questionnaire Q-014-002-M3
Return				

Reason for delay, missed contact efforts or reschedule	No delay. Return contact on 9 Oct only to finish the ASES (I-1).

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Required components

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PFVR ID	Participant code	Visit	Window	Sheet no. / total sheets
PFVR-014-002-M3	014-002	3 months	22 Sep to 20 Oct 2026	1 of 1

D Required components

One line per component the CIP requires for this visit, taken from the visit schedule. Completion and timing are rated separately: a component can be complete but outside its window. Partial, not performed or outside the window needs an exception in I.

#	Component	Critical to quality?	Method required · used	Date · day from index	Completion	Timing	Evidence or record (IQL, questionnaire, note)	Exception (# in I)
1	Constant score	☑ Yes ☐ No	Clinician-administered, dynamometer · as required	2 Oct · day 86	☑ Complete ☐ Partial ☐ Not performed ☐ N/A	☑ In window ☐ Outside window ☐ N/A	Score sheet Q-014-002-M3	
2	ASES questionnaire	☑ Yes ☐ No	Self-completed · as required	2 and 9 Oct · day 86, 93	☑ Complete ☐ Partial ☐ Not performed ☐ N/A	☑ In window ☐ Outside window ☐ N/A	Q-014-002-M3	1
3	Pain score 0 to 10	☑ Yes ☐ No	Self-reported · as required	2 Oct · day 86	☑ Complete ☐ Partial ☐ Not performed ☐ N/A	☑ In window ☐ Outside window ☐ N/A	Q-014-002-M3	
4	X-ray: true AP, Velpeau axillary, scapular Y	☑ Yes ☐ No	SIRR IMG-05 · as required	2 Oct · day 86	☑ Complete ☐ Partial ☐ Not performed ☐ N/A	☑ In window ☐ Outside window ☐ N/A	IQL-014-024	
5	Range of motion	☐ Yes ☑ No	Goniometer · as required	2 Oct · day 86	☑ Complete ☐ Partial ☐ Not performed ☐ N/A	☑ In window ☐ Outside window ☐ N/A	CN-014-002-M3	
6	Interval safety review	☐ Yes ☑ No	Interview and examination · as required	2 Oct · day 86	☑ Complete ☐ Partial ☐ Not performed ☐ N/A	☑ In window ☐ Outside window ☐ N/A	CN-014-002-M3; SART-014-003	2
7		☐ Yes ☐ No			☐ Complete ☐ Partial ☐ Not performed ☐ N/A	☐ In window ☐ Outside window ☐ N/A		
8		☐ Yes ☐ No			☐ Complete ☐ Partial ☐ Not performed ☐ N/A	☐ In window ☐ Outside window ☐ N/A		

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Image and test usability, participant report

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PFVR-014-002-M3	014-002	3 months	22 Sep to 20 Oct 2026

E Images and objective tests

Only for components in D that produce an image or a test result. The image-set quality itself is judged in the IQL.

# in D	Record (IQL or test ID)	Usable?	Reviewer · date	Finding that matters for the visit	Action or route
4	IQL-014-024	☑ Adequate ☐ Limited ☐ Unavailable	Dr H. Meyer · 3 Oct	Implant seated; humeral head centered; no dislocation	None
		☐ Adequate ☐ Limited ☐ Unavailable			
		☐ Adequate ☐ Limited ☐ Unavailable			

F What the participant reports

The participant's own words, including improvement. A report is not classified as an adverse event here; send it for assessment when needed.

Reported change or concern	Onset · severity	Since last visit	Action or referral · record
"It feels like it slips out at the back when I reach behind me."	About 2 weeks; twice a week; moderate	☐ Better ☑ Worse ☐ Same	Seen by the PI the same day; SART-014-003 opened (I-2)
"The pain is less than at six weeks."	3 / 10 now; 5 / 10 at 6 weeks	☑ Better ☐ Worse ☐ Same	None
"I can put a cup on the kitchen shelf."	Since about 3 weeks	☑ Better ☐ Worse ☐ Same	None
		☐ Better ☐ Worse ☐ Same	

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Clinical assessment, personnel and exceptions

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G Clinical assessment

The result and how it compares with the last visit; the full note stays in the clinical source.

Assessment	Assessor	Method	Result · compared with last visit	Action · record
Stability	Dr L. Ortiz	Apprehension test	Positive in internal rotation; new since 6 weeks	PI review 2 Oct; SART-014-003
Range of motion	Dr L. Ortiz	Goniometer	Forward elevation 120 degrees (6 weeks: 90)	None
Wound	Dr L. Ortiz	Inspection	Healed; unchanged	None
Axillary nerve	Dr L. Ortiz	Sensation and deltoid	Intact; unchanged	None

H Who did the assessments

A change of person matters when a measure depends on the assessor; note the handover.

Role	Name	Same as last visit?	Handover or supervision note
Imaging	Elena Martin	☑ Yes ☐ No ☐ N/A	
Clinical assessor	Dr Lena Ortiz	☑ Yes ☐ No ☐ N/A	
Questionnaires and scores	Grace Park	☐ Yes ☑ No ☐ N/A	N. Costa on leave on 2 Oct; G. Park, trained on Constant scoring (TRE-014-09), scored under Dr L. Ortiz

I Exceptions and response

Only for a problem that needs a response: a partial, missing or late component, an unusable result, or a finding that needs a referral. Safety and deviation assessments stay in their own records.

#	Component (# in D) · cause	Effect on the participant or the data · response	Linked record	Owner	Due	Status
1	2: ASES page 2 not done on 2 Oct; participant had to leave	Completed 9 Oct, day 93, in window; no data lost	Q-014-002-M3	Research nurse	9 Oct	☐ Open ☑ Closed
2	6 and F-1: new instability symptoms	PI review; SART-014-003 (grade I); physiotherapy for stabilization; event indexed in SCML-014-09	SART-014-003; SCML-014-09	PI	16 Oct	☐ Open ☑ Closed
3						☐ Open ☐ Closed

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Visit outcome and sign-off

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J Visit outcome

Write what confirms each point (counts, days, item numbers). The classification comes from C to I; an exception does not make a visit unusable by itself.

Point	Result	Confirmed in (section and what was checked)
Visit in its window	☑ Yes ☐ No	C: visit 2 Oct, 4 days before target, in window
Every required component complete	☑ Yes ☐ No	D: 6 of 6 complete
Every critical-to-quality component in window and usable	☑ Yes ☐ No	D and E: 4 of 4 in window; X-ray adequate (IQL-014-024)
No open exception	☑ Yes ☐ No	I: 2 of 2 closed

Outcome	Meaning	Select
Complete and usable	All components complete, in window and usable.	☐
Complete, with a documented observation	Complete and usable; an observation in I does not limit the data.	☑
Usable with a limitation	A component is late, partial or limited; the limit is stated in I.	☐
Incomplete: follow-up required	A component is still missing and can be recovered.	☐
Not usable for the intended assessment	A critical-to-quality component cannot be used.	☐
Missed	The visit did not take place.	☐

Rationale	All components complete and in window; the ASES finished on a second contact inside the window. New instability symptoms were sent to the PI and the SART; they are a clinical finding, not a gap in the visit data.

K Sign-off

Completed by

Name / role: Nadia Costa / Research nurse

Signature / date:

N. Costa 9 Oct 2026

Reviewed by (investigator)

Name: Dr Alex Morgan

Signature / date:

A. Morgan 12 Oct 2026